Neratinib · extended adjuvant treatment

Neratinib is taken once daily with food for one year. The usual starting dose is 240 mg; the team may interrupt or reduce it for toxicity. Treatment starts less than one year after completing adjuvant trastuzumab.

Make treatment fit into daily life

Take tablets whole with food, preferably in the morning. Do not crush or dissolve them. Do not replace a missed dose. Start the prescribed loperamide prevention plan with the first neratinib dose.

Before the first tablet

Confirm the one-year plan, loperamide prescription, blood-test appointments and interaction review.

Daily treatment at home

Take with food, preferably at the same morning time. No infusion port or pump is required for neratinib itself.

Acid-suppression timing

Avoid PPIs. If needed, the pharmacist specifies H2-blocker or antacid timing and checks interactions.

After the planned year

Confirm the stop date; endocrine therapy and early-breast surveillance may continue on their own schedules.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00720 v2 · official regimen
- ESMO 2024 early breast cancer guideline · treatment and follow-up
- ESC 2022 cardio-oncology guideline · cardiac surveillance and risk
- HSE · chemotherapy side effects and urgent advice
- NICE NG101 · bone health and follow-up
- NOGG · section 7, aromatase inhibitors and fracture risk
- eviQ · implanted port patient information
- eviQ · peripherally inserted central catheter (PICC) patient information
- NCCP NCIS antiemetics v8 (2025), Table 1
- ASCO · biomarkers in metastatic breast cancer
- ESMO metastatic breast cancer guideline · 2026
- NCCP oral antiemetic classification v6 · Tables 5 and 8
- EMA Nerlynx product information

Source reconciled; independent clinical, pharmacy and patient/PPI review pending